

HIGHLIGHTS OF PRESCRIBING INFORMATION

These highlights do not include all the information needed to use AMLOZA safely and effectively. See full prescribing information for AMLOZA.

AMLOZA (amlodipine besylate) tablets, for oral use

Initial U.S. Approval: 1987

INDICATIONS AND USAGE

AMLOZA is a dihydropyridine calcium channel blocker indicated for: (1)

- Hypertension, to lower blood pressure, alone or with other antihypertensive agents (1.1)
- Coronary Artery Disease: chronic stable angina and vasospastic (Prinzmetal's) angina (1.2)

DOSAGE AND ADMINISTRATION

- Adults: Initial dose 5 mg once daily; maximum 10 mg once daily (2.1)
- Small, fragile, or elderly patients, or hepatic impairment: initiate at 2.5 mg once daily (2.1)
- Pediatric (6–17 years): 2.5–5 mg once daily (2.2)
- Coronary Artery Disease: 5–10 mg once daily (2.3)

DOSAGE FORMS AND STRENGTHS

Tablets: 5 mg and 10 mg (3)

CONTRAINDICATIONS

Known hypersensitivity to amlodipine besylate or any inactive ingredient (4)

WARNINGS AND PRECAUTIONS

- Hypotension: possible, particularly with severe aortic stenosis (5.1)
- Increased Angina/MI: rare reports on initiation or dose increase in severe obstructive coronary artery disease (5.2)
- Hepatic Impairment: titrate slowly and monitor closely (5.3)

See 17 for *PATIENT COUNSELING INFORMATION*.

FULL PRESCRIBING INFORMATION: CONTENTS

1. INDICATIONS AND USAGE

AMLOZA is a dihydropyridine calcium channel blocker indicated for: hypertension, to lower blood pressure, alone or with other antihypertensive agents; and coronary artery disease — chronic stable angina and vasospastic (Prinzmetal's) angina, alone or in combination with other antianginal agents.

2. DOSAGE AND ADMINISTRATION

2.1 Hypertension — Adults: Initial dose 5 mg once daily; maximum 10 mg once daily. Small, fragile, or elderly patients, or patients with hepatic impairment: initiate at 2.5 mg once daily. 2.2 Pediatric Hypertension (6–17 years): 2.5–5 mg once daily; doses above 5 mg have not been studied. 2.3 Coronary Artery Disease: 5–10 mg once daily; most patients require 10 mg. 2.4 No initial dosage adjustment required in renal impairment.

3. DOSAGE FORMS AND STRENGTHS

Tablets: 5 mg and 10 mg (as amlodipine).

4. CONTRAINDICATIONS

Known hypersensitivity to amlodipine besylate or any inactive ingredient.

5. WARNINGS AND PRECAUTIONS

5.1 Hypotension: Symptomatic hypotension is possible, particularly in patients with severe aortic stenosis. Use with caution. 5.2 Increased Angina/MI: Rare cases of increased frequency, duration, or severity of angina or acute myocardial infarction have been reported on starting or increasing the dose, particularly in patients with severe obstructive coronary artery disease. 5.3 Hepatic Impairment: Amlodipine is extensively metabolized by the liver; elimination half-life is prolonged in hepatic impairment. Titrate slowly and monitor closely.

6. ADVERSE REACTIONS

6.1 Clinical trial experience: Most common (>1%, more than placebo): edema (10.8%), dizziness (3.4%), flushing (2.6%), palpitation (1.8%), headache. Edema is dose-related and more common in women. 6.2 Postmarketing: Gynecomastia, jaundice, hepatic enzyme elevation (mostly cholestasis), erythema multiforme have been reported rarely.

7. DRUG INTERACTIONS

7.1 CYP3A Inhibitors: Concomitant use with strong and moderate CYP3A inhibitors increases amlodipine exposure; monitor for hypotension and edema. 7.2 CYP3A Inducers: May decrease amlodipine plasma concentration; monitor blood pressure. 7.3 Simvastatin: Concomitant use increases simvastatin exposure; limit simvastatin dose to 20 mg daily. 7.4 Tacrolimus/Cyclosporine: May increase levels; monitor concentrations.

8. USE IN SPECIFIC POPULATIONS

8.1 Pregnancy: Limited human data; use only if potential benefit justifies potential risk to fetus. 8.2 Lactation: Present in human milk; weigh benefits against potential infant risk. 8.4 Pediatric (6–17 years): Safety and effectiveness for hypertension established. 8.5 Geriatric: Amlodipine clearance may be reduced — lower starting dose reasonable. 8.6 Hepatic Impairment: Half-life prolonged; use lowest starting dose and titrate slowly.

10. OVERDOSAGE

Overdose may be expected to cause excessive peripheral vasodilation with marked hypotension and possibly reflex tachycardia. In humans, experience with intentional overdose is limited. If massive overdose occurs, initiate active cardiac and respiratory monitoring. Frequent blood pressure measurements are essential. Clinically significant hypotension due to amlodipine overdose requires active cardiovascular support, including elevation of the extremities and judicious administration of fluids. Vasopressors may be helpful in restoring vascular tone and blood pressure if there is no contraindication to their use. Intravenous calcium gluconate may help reverse the effects of calcium channel blockade. Because amlodipine is highly protein bound, hemodialysis is not likely to be beneficial.

11. DESCRIPTION

Amlodipine besylate is the besylate salt of amlodipine, a long-acting calcium channel blocker of the dihydropyridine class. Molecular formula: $C_{20}H_{25}ClN_2O_5 \cdot C_6H_6O_3S$; Molecular weight: 567.1. Each AMLOZA tablet contains amlodipine besylate equivalent to 5 mg or 10 mg amlodipine. AMLOZA tablets are white, round, biconvex, uncoated tablets, consistent with the description in Module 3.2.P.1.

12. CLINICAL PHARMACOLOGY

12.1 Mechanism of action: Inhibits calcium ion influx across cardiac and vascular smooth muscle cell membranes, producing coronary and peripheral vasodilation, reducing peripheral vascular resistance and blood pressure. 12.3 Pharmacokinetics: Slowly absorbed; peak plasma concentrations 6–12 hours post-dose. Absolute bioavailability 64–90%. Approximately 93% plasma protein bound. Extensively metabolized in the liver to inactive metabolites. Elimination half-life approximately 30–50 hours, consistent with once-daily dosing.

16. HOW SUPPLIED / STORAGE

5 mg and 10 mg tablets, supplied in bottles of 30, 90, and 500 tablets (NDC 12345-678-90 (placeholder)). Store at 25°C (77°F); excursions permitted to 15°C–30°C (59°F–86°F).

17. PATIENT COUNSELING INFORMATION

AMLOZA may be taken with or without food. Report swelling of the ankles/feet, palpitations, or worsening chest pain promptly. Avoid abrupt discontinuation without consulting the prescriber. Inform the prescriber of all concomitant medications, particularly simvastatin or strong CYP3A inhibitors.

Portfolio Note: This PI follows the Physician Labeling Rule (PLR) under 21 CFR 201.56 and 201.57. In a real ANDA, labeling must match the approved RLD (Norvasc) with only generic-specific administrative modifications permitted.

Revised: 07/2026 (Academic Mock ANDA Portfolio) — this revision date is illustrative and does not represent an official FDA labeling revision or approval action.